THE Open GROUP

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Barangay San Antonio Health Department Enterprise Architecture

Barangay San Antonio Health Center, Pasig City

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Tracking Information

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1. Purpose of this Document

The Architecture Repository acts as a holding area for all architecture-related projects within the enterprise. The repository allows projects to manage their deliverables, locate re-usable assets, and publish outputs to stakeholders and other interested parties.

For this engagement, the Architecture Repository is the GitHub repository that holds every deliverable, diagram and supporting record of the Barangay San Antonio Health Department study (ADM Preliminary Phase to Phase E). This document indexes what is stored where, using the repository areas defined by the TOGAF Standard.

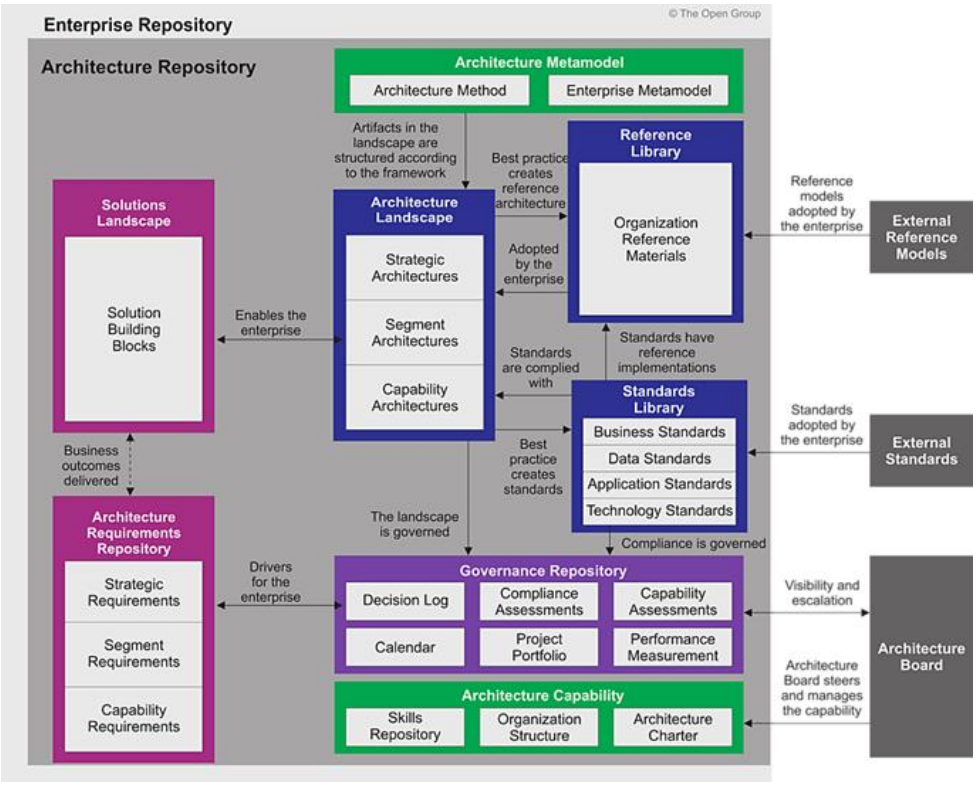
Repository folder	What it holds
Barangay San Antonio Health Department - Phase A (Architecture Vision) … Barangay San Antonio Health Department - Phase E (Opportunities and Solutions)	The ADM deliverables of each phase (DOCX and PDF), each with a Diagrams subfolder holding that phase's diagrams as images
Barangay San Antonio Health Department - Architecture Repository	This document plus the TOGAF repository areas below, each with a README describing its contents
Barangay San Antonio Health Department - ADM Deliverables Checklist.xlsx	Checklist of every deliverable, artifact and diagram, with file name, location and status

1.1 Output and Input

The TOGAF® Content Framework (see Architecture Deliverables) identifies deliverables that are produced as outputs from executing the Architecture Development Method (ADM) cycle and potentially consumed as inputs at other points in the ADM.

This deliverable is produced, updated, and consumed by the ADM Phases:

Deliverable	Output from…	Input to…
Architecture Vision	Preliminary	Preliminary, A, B, C, D, E, F, G, H, Requirements Management


Enterprise Repository
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Architecture Repository
Architecture Metamodel
Architecture Method
Enterprise Metamodel
Artifacts in the landscape are structured according to the framework
Best practice creates reference architecture
Reference Library
Organization Reference Materials
Reference models adopted by the enterprise
External Reference Models
Solutions Landscape
Solution Building Blocks
Enables the enterprise
Architecture Landscape
Strategic Architectures
Segment Architectures
Capability Architectures
Adopted by the enterprise
Standards are complied with
Standards have reference implementations
Standards Library
Business Standards
Data Standards
Application Standards
Technology Standards
Standards adopted by the enterprise
External Standards
Business outcomes delivered
Best practice creates standards
The landscape is governed
Compliance is governed
Architecture Requirements Repository
Strategic Requirements
Segment Requirements
Capability Requirements
Drivers for the enterprise
Governance Repository
Decision Log
Compliance Assessments
Capability Assessments
Calendar
Project Portfolio
Performance Measurement
Visibility and escalation
Architecture Board
Architecture Board steers and manages the capability
Architecture Capability
Skills Repository
Organization Structure
Architecture Charter

2. Architecture Landscape

The Architecture Landscape holds architectural views of the enterprise at three levels of detail.

2.1 Strategic Architectures

Out of scope. The strategic level is the Pasig City health system owned by the City Health Department (City EMR, City-wide procurement, City staffing). It is referenced only as a constraint: the center cannot change City- or nationally owned systems, and the target architecture keeps the City EMR as the system of record.

2.2 Segment Architectures

This engagement is a segment architecture: one health center within the Pasig City health landscape. Its baseline and target are recorded in:

- A5 Architecture Definition (Barangay San Antonio Health Department - Phase A folder): baseline, target and transition architectures across all four domains
- B1 Architecture Deliverables – Phase B: business catalogs, matrices and process flows
- C1 Architecture Deliverables – Phase C: data and application artifacts
- D1 Architecture Deliverables – Phase D: technology artifacts

2.3 Capability Architectures

Capability-level views are the nine work packages (WP1–WP9) and their building blocks: E2 Architecture Building Blocks (ABB-1 to ABB-9) and E3 Solution Building Blocks (SBB-1 to SBB-9), sequenced in E1 Architecture Roadmap.

3. Reference Library

Reference material used to develop the architecture.

3.1 Overview

The Reference Library holds the external guidance, models and templates the team used. Copies of our own briefing material are kept in the Governance Repository folder.

3.2 Standards Bodies

- The Open Group: TOGAF® Standard (ADM, Content Metamodel, templates)
- Department of Health (DOH): FHSIS, program guidelines, records retention
- National Nutrition Council (NNC): Philippine Plan of Action for Nutrition, Operation Timbang Plus
- PhilHealth: YAKAP and GAMOT rules
- National Privacy Commission: Data Privacy Act (RA 10173) implementation

3.3 Product and Service Vendors

Product or service	Provider	Role in the architecture
City EMR (eMed) and dental eMED	City of Pasig, through an outsourced vendor	System of record; unchanged by this project
PhilHealth HCI portal / YAKAP	PhilHealth	Registration, prescriptions, claims
LMIS, ITIS, FHSIS	DOH / City	Logistics, TB and statistics reporting
e-OPT Plus tool	National Nutrition Council	OPT Plus data entry
Internet line	PLDT (barangay-paid)	Single connection for all cloud systems
Google Drive, Messenger, Viber	Google, Meta, Rakuten Viber	Currently used on personal accounts; WP2 moves work to an official workspace

3.4 Industry Communities or Forums

None engaged. Academic guidance came from the UA&P Information Technology program (Mr. Joel Pira, instructor; Dr. Amado Salvador P. Saquido, Program Director).

3.5 Standard Templates

The TOGAF® template pack provided in the course (folders Preliminary Phase to Phase H). Templates used: Architecture Vision, Statement of Architecture Work, Capability Assessment, Communications Plan, Architecture Definition, Architecture Requirements Specification, Architecture Roadmap, Architecture Deliverables Phases C and D, Implementation and Migration Plan, Architecture Building Blocks, Solution Building Blocks, and this Architecture Repository template.

3.6 Enterprise Best Practice

- Buddy system for staff coverage (health center practice)
- Monthly program reports submitted on time, with no late reports recorded
- Paper fallback during internet outages
- DOH 7-year retention with a disposal logbook naming the collector

3.7 Reference Architectures

No external reference architecture was adopted. The target architecture is shaped by the three architecture principles (A5, Section 2.3): One Patient, One Record; Use the City Record, Don't Rebuild It; Report Once, Count Many Times.

3.8 Reference Models

TOGAF Content Metamodel, tailored to 34 entities: 26 retained, 3 deleted (Product, Contract, Course of Action), 7 merged, and 8 added (Individual Treatment Record, Consultation Encounter, Consent Record, Health Program, Monthly Program Report, GAMOT Dispensing Entry, Referral, Record Retention and Disposal Entry). See the Architecture Metamodel folder.

3.9 Viewpoint Library

Viewpoint (diagram)	Phase	Stored in
Stakeholder Map	A	Barangay San Antonio Health Department - Phase A … / Diagrams
Value Chain Diagram	A	Barangay San Antonio Health Department - Phase A … / Diagrams
Solution Concept Diagram	A	Barangay San Antonio Health Department - Phase A … / Diagrams
Business Capability Map	B	Barangay San Antonio Health Department - Phase B … / Diagrams
Process Flow Diagrams (BP-1, BP-3, BP-5)	B	Barangay San Antonio Health Department - Phase B … / Diagrams
Logical Data Diagram	C	Barangay San Antonio Health Department - Phase C … / Diagrams
Data Lifecycle Diagram	C	Barangay San Antonio Health Department - Phase C … / Diagrams
Application Communication Diagram	C	Barangay San Antonio Health Department - Phase C … / Diagrams
Environments and Locations Diagram	D	Barangay San Antonio Health Department - Phase D … / Diagrams
Architecture Roadmap	E	Barangay San Antonio Health Department - Phase E … / Diagrams

3.10 Templates

Naming convention used throughout the repository: folders are named "Barangay San Antonio Health Department - Phase X (Phase name)" and files "Barangay San Antonio Health Department - <code> -

<document name>", for example "Barangay San Antonio Health Department - A1 - Architecture Vision.pdf".

4. Standards Library

Standards the architecture must comply with.

4.1 Overview

The standards below were identified from the interviews and verified with the health center. They are mandatory unless stated otherwise.

4.2 Legal and Regulatory

- Data Privacy Act of 2012 (RA 10173): consent on patient records; restricted access to sensitive records such as mental health
- DOH records retention: 7 years, with disposal logged

4.3 Industry

- PhilHealth YAKAP / GAMOT rules for registration, prescriptions (UPSC, QR code) and claims
- DOH FHSIS reporting
- NNC Philippine Plan of Action for Nutrition and OPT Plus validity (at least 80% coverage)
- DOH sterilizer standard for dental accreditation

4.4 Organizational

- Minor process or system changes need City Health Officer approval; major ones need the City Mayor
- The health center cannot install software; the City approves and trains for new systems
- City equipment audited about every 6 months; barangay equipment checked every 3 months
- Repository naming convention (Section 3.10)

4.5 Standards Classification

Standard	Type	Domain	Status
Data Privacy Act (RA 10173)	Legal	Data, Application	Mandatory
DOH 7-year records retention	Legal	Data	Mandatory
PhilHealth YAKAP / GAMOT rules	Industry	Business, Application	Mandatory
DOH FHSIS reporting	Industry	Business, Data	Mandatory
NNC OPT Plus validity (≥80%)	Industry	Business	Mandatory
DOH sterilizer standard	Industry	Technology	Mandatory
City approval for system changes	Organizational	All	Mandatory
City software-installation rule	Organizational	Application, Technology	Mandatory
Equipment audit schedule	Organizational	Technology	Mandatory

4.6 Business Standards

Program reporting cycle: data from the 30th, consolidated by the 1st, reviewed by the City Health Department before the first Wednesday; signed hard copies currently required. OPT Plus runs January to March.

4.7 Data Standards

Data Privacy Act consent and access rules; 7-year retention; the EMR record number as the single patient key (principle One Patient, One Record).

4.8 Application Standards

The City EMR remains the system of record; any center-level tool must feed it and must not create a second patient database (principle Use the City Record, Don't Rebuild It).

4.9 Technology Standards

Software installation only with City approval; individual EMR accounts; device audits on the City and barangay schedules.

5. Governance Repository

Records of how the engagement was approved, decided and communicated.

5.1 Overview

The Governance Repository folder holds the permit and request letters, the signed Received By sheet, and the briefing materials shared with the health center.

5.2 Decision Log

Date (2026)	Decision	By
September	Scope set to all operations of the health department; tailored framework and three architecture principles adopted	Team (Preliminary Phase)
October 15	Permit request to interview personnel submitted through Ms. Nita N. Hardman, RN, to the City Health Officer; approved	City Health Officer
October	OIC health aide, utility staff and barangay IT team not interviewed	Team
October 22	Where interview figures and the health center's written answers differ, the health center's answers are used	Team
October 22	Deliverables rebuilt on the course TOGAF templates; Architecture Deliverables template used for Phase B; Roadmap and building blocks filed in Phase E (tailoring notes in B1, E1, E2, E3)	Team
October 23	Presentation of findings and roadmap to the Executive Sponsor scheduled; she asked for time after the presentation before deciding on sign-off	Dr. Glynnis Lynn U. Bonilla
November	Submission moved from Google Drive to a private GitHub repository, as instructed	Mr. Joel Pira

5.3 Compliance Assessments

None yet. Compliance assessments belong to Phase G (Implementation Governance), which is outside this engagement.

5.4 Capability Assessments

A3 Capability Assessment rates ten business capabilities from 1 to 5. Weakest (rated 2, target 4): medicine logistics, records management, and IT and equipment.

5.5 Calendar

When	Event
Mon, Tue, Fri 8–11 AM	Consultations
Mon, Wed, Fri afternoons	Maintenance-medicine pickups
30th to 1st of each month	Program report compilation
November 2026	2027 barangay budget proposals due
November–December 2026	Roadmap stage T1: quick wins
January–March 2027	OPT Plus and MR-SIA: no rollouts

When	Event
April–June 2027	Roadmap stage T2: equip and digitize
July–December 2027	Roadmap stage T3: integrate and extend

5.6 Project Portfolio

Project	Work packages	Sponsor
P1 Medicine Assurance	WP1	Kagawad Rustia-Olarte; Nurse Hardman
P2 Digital Workplace	WP2, WP3, WP5, WP7	Kapitan Lising
P3 Records Modernization	WP4	Dr. Bonilla
P4 Patient Flow	WP6	Dr. Bonilla; UA&P IT program
P5 City Interoperability	WP8	Dr. Bonilla; Kapitan Lising
P6 Community Reach	WP9	Kagawad Rustia-Olarte

Source: E4 Implementation and Migration Plan.

5.7 Performance Measurement

Success measures SM-1 to SM-8 are defined in B2 Architecture Requirements Specification, for example: prescription entries per consultation (up to 3 → 1), months with maintenance-medicine stock-outs (3 in 2026 → 0), and old-patient record retrieval (2–10 minutes → under 1 minute).

6. Solutions Landscape

Current solutions in use: City EMR (eMed), dental eMED, PhilHealth HCI / YAKAP, LMIS, ITIS, NNC e-OPT Plus, and Google Drive and chat apps on personal accounts (see C1, Application Portfolio Catalog).

Proposed solutions: Solution Building Blocks SBB-1 to SBB-9 in E3, each realizing one Architecture Building Block in E2.

7. The Enterprise Repository

The enterprise repository for this engagement is a private GitHub repository shared with the course instructor. Phase folders hold the deliverables and diagrams; the "Barangay San Antonio Health Department - Architecture Repository" folder holds this document and the TOGAF repository areas; the checklist workbook at the top level tracks completeness.

8. External Repositories

External sources referenced by the architecture.

8.1 External Reference Models

TOGAF® Standard, 10th Edition (The Open Group).

8.2 External Standards

DOH, NNC, PhilHealth and National Privacy Commission issuances (see Standards Library).

8.3 Architecture Board Approvals

There is no formal Architecture Board at the health center. Approvals recorded: City Health Officer permit for interviews (October 2026). Executive Sponsor sign-off of the Architecture Vision: pending. A small change board (Dr. Bonilla, Kagawad Rustia-Olarte, Nurse Hardman) is proposed in A3.

9. Architecture Capability

The people, roles and charter behind the architecture work.

9.1 Skills Repository

Group	Skills relevant to the architecture
UA&P team (four students)	TOGAF ADM, interviewing, modeling, documentation
Clinical staff and encoder	Digital skills about 5/5 (doctor's estimate)
Pasig Health Aides and BHWs	Digital skills about 2/5; training planned in WP3

9.2 Organization Structure

Role	Person
Executive Sponsor	Dr. Glynnis Lynn U. Bonilla, Medical Officer IV
Barangay sponsors	Hon. Thomas Raymond U. Lising; Hon. Rachel Marie S. Rustia-Olarte
Coordination	Ms. Nita N. Hardman, RN; Ms. Joan Marie E. Barcenas
Group Leader / Lead Architect	Rome Victor Mag-iba Sendin
Architects	Samuel Anthonius Badillo; Wilson Halim; Kevin Enrico Ang
Academic oversight	Mr. Joel Pira; Dr. Amado Salvador P. Saquido

9.3 Architecture Charter

Purpose: study all operations of the Barangay San Antonio Health Department and deliver a roadmap the barangay and health center can act on. Scope and acceptance are defined in A2 Statement of Architecture Work. The architecture is governed by three principles: One Patient, One Record; Use the City Record, Don't Rebuild It; Report Once, Count Many Times.

A Abbreviations

Abbreviations, acronyms and reference codes used in this document, in alphabetical order.

Abbreviation	Meaning
ABB	Architecture Building Block (ABB-1 to ABB-9 are reference numbers)
ADM	Architecture Development Method, the step-by-step method of the TOGAF Standard
BHW	Barangay Health Worker
BP-1 …	Business process reference number
DOH	Department of Health
e-OPT	Electronic Operation Timbang Plus tool of the National Nutrition Council
eMed / eMED	Name of the City's electronic medical record system; “dental eMED” is its separate dental system
EMR	Electronic Medical Record
FHSIS	Field Health Services Information System (DOH)
GAMOT	PhilHealth Guaranteed and Accessible Medications for Outpatient Treatment
HCI	Health Care Institution (PhilHealth portal for health facilities)
Hon.	Honorable (title of elected officials)
IT	Information technology
ITIS	Integrated Tuberculosis Information System (DOH)
LMIS	Logistics Management Information System
MR-SIA	Measles-Rubella Supplemental Immunization Activity
NNC	National Nutrition Council
OIC	Officer-in-Charge
OPT / OPT Plus	Operation Timbang Plus: yearly weighing and measuring of children aged 0–59 months
PhilHealth	Philippine Health Insurance Corporation
PLDT	PLDT Inc., an internet and telephone provider
QR	Quick Response (a scannable square barcode)
RA	Republic Act (a Philippine law)
RN	Registered Nurse
SBB	Solution Building Block (SBB-1 to SBB-9 are reference numbers)
SM-1 …	Success measure reference number
T1 … T3	Transition architecture stage
TB	Tuberculosis
TOGAF	The Open Group Architecture Framework (the TOGAF® Standard)
UA&P	University of Asia and the Pacific
UPSC	Unique Prescription Security Code
WP1 … WP9	Work package (a group of related changes delivered together)
YAKAP	PhilHealth Yaman ng Kalusugan Program (primary-care benefit)